PT Nirwana Bhumi Energy

OCCUPATIONAL DISEASE REPORTING AND INVESTIGATION PROCEDURE

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REVISION INDEX

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TABLE OF CONTENTS

1.0 OBJECTIVE
2.0 SCOPE
3.0 DEFINITIONS & ABBREVIATIONS
4.0 RESPONSIBILITIES
5.0 PROCEDURE STEPS
6.0 PERFORMANCE INDICATORS & AUDIT
7.0 REFERENCED DOCUMENTS
8.0 ATTACHMENTS & ASSOCIATED FORMS

1.0 OBJECTIVE

The objective of this procedure is to establish a standardized, transparent, and legally compliant framework for the early detection, formal notification, clinical diagnosis, root-cause investigation, and statutory reporting of all suspected or confirmed Occupational Diseases (Penyakit Akibat Kerja - PAK) affecting personnel of PT Nirwana Bhumi Energy.

Specifically, this procedure guarantees that:

- Workplace health hazards causing occupational illnesses are systematically investigated and eliminated using the hierarchy of controls.
- Affected workers receive immediate clinical care, specialized occupational health referral, and statutory workers' compensation protection via BPJS Ketenagakerjaan.
- Full compliance is maintained with Presidential Regulation No. 7/2019, Minister of Manpower Regulation No. Per.01/MEN/1981, and energy sector CSMS standards.

2.0 SCOPE

This procedure applies to all operational sites, fabrication workshops, warehouses, and client field projects of PT Nirwana Bhumi Energy.

The scope covers:

- All full-time employees, contract personnel, apprentices, and third-party contractor workers under company supervision.
- All categories of occupational illnesses recognized under statutory legislation, including respiratory diseases, occupational dermatitis, noise-induced hearing loss (NIHL), musculoskeletal disorders (MSDs), toxic chemical poisonings, and physical agent disorders.

3.0 DEFINITIONS & ABBREVIATIONS

Occupational Disease (Penyakit Akibat Kerja - PAK): Any disease or health impairment caused directly by work activities, exposure to hazardous substances, or physical/ergonomic factors present in the working environment.

Coordinating Doctor (Dokter Koordinator): A certified occupational health physician appointed to evaluate clinical diagnoses, investigate workplace etiology, and determine occupational causality.

Investigation Committee: A multidisciplinary team comprising the HSE Coordinator, Company Doctor, Area Supervisor, and Worker Representative tasked with uncovering underlying root causes.

BPJS Ketenagakerjaan: The statutory Indonesian social security agency administering employment injury and occupational illness compensation.

4.0 RESPONSIBILITIES

Director of Operations & Technology / CTO: Authorizes and enforces immediate engineering and operational modifications required to eliminate disease-causing hazards at work sites.

Director of Finance & Human Resources / CFO: Oversees employee healthcare administration, statutory compensation claim filings with BPJS Ketenagakerjaan, and return-to-work job accommodations.

HSE Coordinator / P2K3 Secretary: Leads the occupational disease investigation, coordinates industrial hygiene measurements, and ensures statutory reporting to Disnaker within mandated deadlines.

Coordinating Doctor / Occupational Physician: Conducts the clinical examination, applies the 7-step occupational diagnosis protocol, and issues formal medical opinions regarding occupational causality.

Work Area Supervisors: Immediately report any employee exhibiting symptoms of occupational illness using Form 0002-FOR-OID-COO-X-2026 within twenty-four (24) hours of notification.

5.0 PROCEDURE STEPS

5.1 Initial Notification Protocol

Whenever an employee experiences health symptoms suspected of being work-related, or when periodic medical surveillance indicates abnormal physiological changes, the supervisor shall immediately complete the Occupational Disease Notification and Report Form (Form 0002-FOR-OID-COO-X-2026).

The notification must be submitted to the HSE Coordinator and Company Medical Doctor within twenty-four (24) hours of detection.

5.2 Seven-Step Occupational Diagnosis Protocol

The Coordinating Doctor shall evaluate the reported case in strict adherence to the internationally recognized Seven-Step Occupational Diagnosis Method:

- Step 1: Clinical Diagnosis — Confirm clinical pathology through physical examination and laboratory/diagnostic tests.
- Step 2: Workplace Exposure Assessment — Identify specific chemical, physical, biological, or ergonomic agents present in the worker's environment.
- Step 3: Plausibility of Exposure-Disease Relationship — Verify evidence-based medical consensus linking the hazard to the diagnosed condition.
- Step 4: Exposure Magnitude and Duration — Quantify cumulative exposure levels against statutory Threshold Limit Values (TLVs / NAB).
- Step 5: Individual Confounding Factors — Evaluate pre-existing conditions, personal lifestyle factors, or medical history.
- Step 6: Non-Occupational Exposures — Rule out external domestic, hobby, or environmental causes.
- Step 7: Final Diagnostic Determination — Formally classify the case as an Occupational Disease (PAK), Work-Aggravated Condition, or Non-Occupational Disease.

5.3 Comprehensive Workplace Investigation

Upon confirmation of a suspected occupational disease, the Investigation Committee shall convene within forty-eight (48) hours to conduct a detailed investigation utilizing Form 0001-FOR-OID-COO-X-2026.

The investigation shall analyze environmental monitoring data (sound levels, airborne concentrations, lux ratings), review PPE usage history, assess engineering control adequacy, and identify underlying organizational deficiencies.

5.4 Implementation of Corrective and Preventive Actions

The HSE Coordinator and Department Lead shall enforce mandatory controls following the Hierarchy of Controls:

- Elimination / Substitution: Replace hazardous solvents or high-toxicity substances with non-toxic alternatives.
- Engineering Controls: Install local exhaust ventilation, acoustic enclosures, or ergonomic lifting aids.
- Administrative Controls: Enforce job rotation to reduce daily exposure duration below statutory permissible limits.
- Personal Protective Equipment: Provide NIOSH/CE-certified specialized respirators, double hearing protection, or chemical barrier suits.

5.5 Statutory Reporting to Government Authorities

In compliance with Minister of Manpower Regulation No. Per.01/MEN/1981, all confirmed cases of occupational disease must be formally reported to the local Manpower Office (Disnaker) and BPJS Ketenagakerjaan within two times twenty-four hours (2 x 24 hours) of official clinical diagnosis, utilizing statutory Form KK2.

5.6 Medical Rehabilitation and Return-to-Work

The Company Doctor and HR Department shall monitor the affected employee's medical recovery. Upon rehabilitation, the employee shall undergo a formal Return-to-Work assessment, facilitating reassignment to a modified or non-exposed workstation where appropriate.

6.0 PERFORMANCE INDICATORS & AUDIT

Effectiveness of occupational illness prevention shall be tracked against:

- 100% of reported cases investigated within 48 hours of initial notification.
- 100% statutory notification compliance within the mandatory 2 x 24 hour legal reporting window.
- Zero recurring cases of occupational disease stemming from the same workplace hazard.

7.0 REFERENCED DOCUMENTS

Law No. 1 of 1970 regarding Occupational Safety

Presidential Regulation No. 7 of 2019 regarding Occupational Diseases

Minister of Manpower and Transmigration Regulation No. Per.01/MEN/1981 regarding Reporting of Occupational Diseases

ISO 45001:2018 Occupational Health and Safety Management Systems - Clause 10.2

0007-SOP-HSE-COO-X-2026: Employee Medical Surveillance and Health Examination Procedure

8.0 ATTACHMENTS & ASSOCIATED FORMS

0001-FOR-OID-COO-X-2026: Occupational Disease Investigation Report Form

0002-FOR-OID-COO-X-2026: Occupational Disease Notification and Report Form